INDIVIDUAL CASE SAFETY REPORT (ICSR)

PHARMACOVIGILANCE & SAFETY DATA MANAGEMENT FORM | E2B (R3) COMPLIANT STANDARD

Safety Report ID:	**PV-2026-US-009482**	**Receipt Date:**	28-SEP-2026
Type of Report:	Spontaneous Post-Marketing	**Initial / Follow-Up:**	Initial (v1.0)
Worldwide Unique ID:	US-COMPANYNAME-2026PV09482	**Report Expedited:**	YES (15-Day Clock)

SECTION A: PATIENT DEMOGRAPHICS & HEALTH PROFILE

Patient Identifier / Initials	Age / DOB	Sex	Weight / Height	Ethnic Origin
J.D. (ID: PT-8834)	58 Years (14-May-1968)	Female	78 kg / 165 cm	Caucasian

Relevant Medical History & Pre-existing Conditions	Relevant Concomitant Medications
• Type 2 Diabetes Mellitus (Diagnosed 2018) • Essential Hypertension (Diagnosed 2015) • Hyperlipidemia (Diagnosed 2020) • Non-Smoker, No documented drug allergies	• Metformin HCl 1000 mg BID (Oral) — Ongoing • Lisinopril 20 mg QD (Oral) — Ongoing • Atorvastatin 20 mg QD (Oral) — Ongoing

SECTION B: SUSPECT PRODUCT INFORMATION

Product Name / Active Substance	Dose & Frequency	Route	Lot / Batch Number	Therapeutic Indication
GlucaTide (Tirzepatide)	5 mg / 0.5 mL (Weekly)	Subcutaneous	BCH-2026-X992	Type 2 Diabetes Mellitus
Date Initiated: 01-AUG-2026		**Date Stopped / Action:** 25-SEP-2026 (Drug Withdrawn)		**Action Taken:** Drug Stopped

SECTION C: ADVERSE DRUG REACTION (ADR) DETAILS

Adverse Event (Verbatim)	MedDRA Term (PT / LLT)	Onset Date & Time	Outcome
Severe upper abdominal pain radiating to back, persistent vomiting	**Acute Pancreatitis** (MedDRA PT: 10000865)	24-SEP-2026 08:30 AM	Recovering / Resolving

Seriousness Criteria Assessment (Check all that apply)

☐ Results in Death ☐ Life-Threatening ■ Requires or Prolongs Inpatient Hospitalization ☐ Persistent/Significant Disability
☐ Congenital Anomaly ☐ Other Medically Important Event

SECTION D: CASE NARRATIVE SUMMARY

Case History & Timeline:
A 58-year-old female patient (J.D.) with a medical history of Type 2 Diabetes Mellitus and Essential Hypertension was prescribed **GlucaTide (Tirzepatide) 5 mg weekly SC** starting on 01-AUG-2026 for glycemic control. Concomitant medications included Metformin 1000 mg BID and Lisinopril 20 mg QD.

On 24-SEP-2026, approximately 7 weeks after commencing treatment, the patient presented to the emergency room with severe, unrelenting epigastric pain radiating to the back, accompanied by nausea and severe vomiting. Laboratory investigations revealed serum lipase levels elevated at 890 U/L (normal range 10-140 U/L) and serum amylase at 410 U/L. Abdominal CT scan

confirmed acute interstitial edematous pancreatitis without necrosis.

Dechallenge / Rechallenge & Outcome:
GlucaTide was permanently discontinued on 25-SEP-2026 (Positive Dechallenge). The patient was hospitalized and treated with IV hydration, analgesics, and bowel rest. As of 28-SEP-2026, the patient's symptoms significantly improved, and serum lipase decreased to 180 U/L. No rechallenge was attempted.

SECTION E: CAUSALITY EVALUATION & REPORTER DETAILS

Causality Scale	Assessed Category	Company Causality	Reporter Causality
WHO-UMC Assessment	**Probable / Likely**	Probable	Suspected Drug Reaction
Naranjo Algorithm Score	**Score: +7 (Probable)**	Temporal relationship clear; positive dechallenge; known reaction for GLP-1/GIP class.	

Primary Reporter Name	Qualification	Institution / Health Center	Contact / Location
Dr. E. Miller, MD	Gastroenterologist	St. Jude University Hospital	Chicago, IL, USA